

AMBIENT ASSESSMENT

Testing Application

Most organizations measure adoption. This measures what adoption produces.

Quality Infrastructure Present. Value Chain Not Yet Built. · Prepared by Abridge

PREPARED FOR

Testing Application

PREPARED BY

Brad Larmie · March 19, 2026

EXECUTIVE SUMMARY

This isn't a summary of your deployment. It's a measurement of what your deployment is producing across four organizational value domains – and a map of what hasn't been measured yet. The numbers that appear below are either confirmed from your stated inputs or labeled as benchmark ranges. Nothing is projected. Nothing is extrapolated without being labeled.

This assessment is based on 60 providers and the activation levels your team reported across four domains. The first number below reflects value the math can confirm. The second reflects what organizations your size typically find when they analyze domains you haven't measured yet.

MATURITY SCORE	CONFIRMED VALUE	UNMEASURED RANGE
55 /100 Acting	\$488K per year · from your data	All domains measured

COST OF A 12-MONTH DELAY

Every year at your current maturity level, the full gap persists. At your scale, waiting 12 months costs between \$488K and \$488K in value not yet captured – the confirmed portion plus the unmeasured range.

Per Year Delayed

\$488K–\$488K

WHAT THIS ASSESSMENT FOUND

Quality Infrastructure Present. Value Chain Not Yet Built.

Documentation quality is being tracked and monitored. The connection from that quality improvement to coding accuracy, CDI, and compliance programs haven't been formalized yet.

- Workforce is your strongest measured domain at \$210K/year – confirmed from your data and stated inputs.
- One to two years in. Every month the unmeasured domains stay unmeasured is a month of value sitting uncounted.

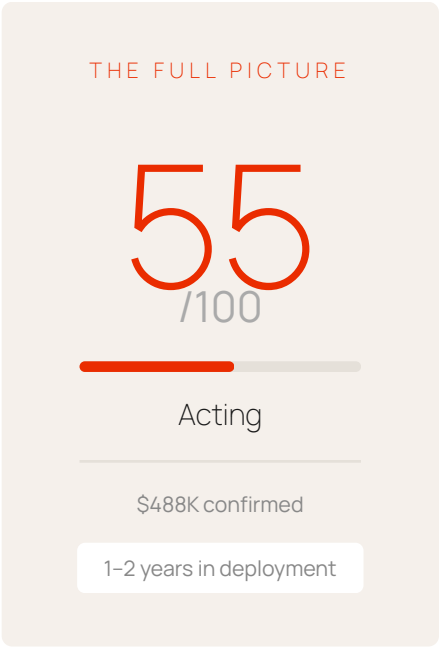
THE DIAGNOSIS

Quality Infrastructure Present. Value Chain Not Yet Built.

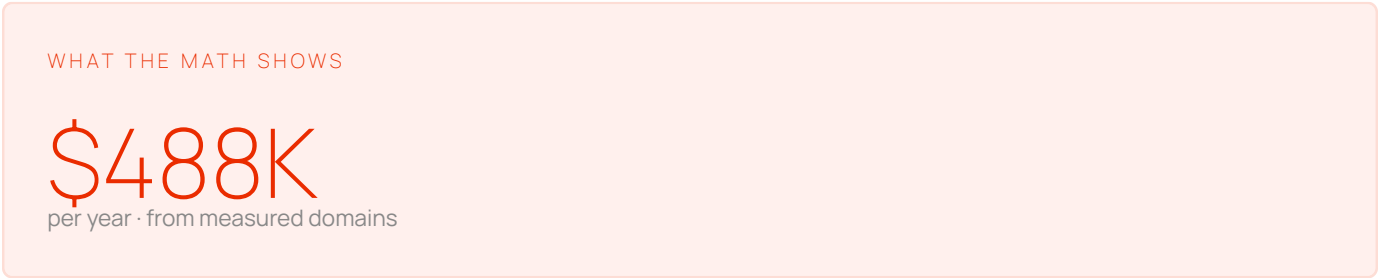
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WHERE THIS PROFILE SITS IN THE ADOPTION CURVE

One to two years in with moderate maturity. Some domains are yielding confirmed value; others haven't been analyzed. At this stage, the gap isn't about adoption – it's about whether there's a structured program to capture what's already generating returns.



WHAT THIS ASSESSMENT FOUND



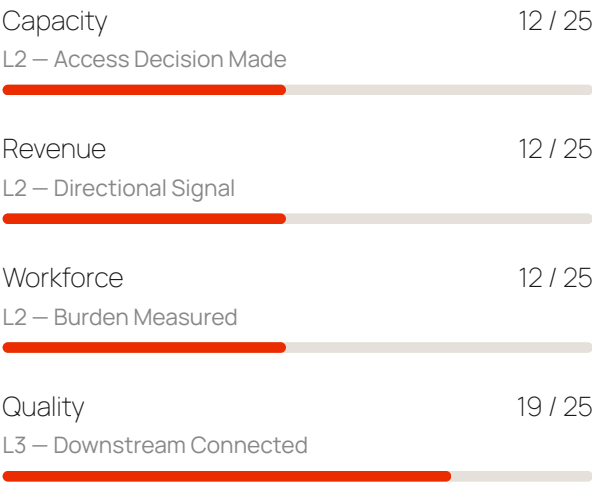
MATURITY POSITION



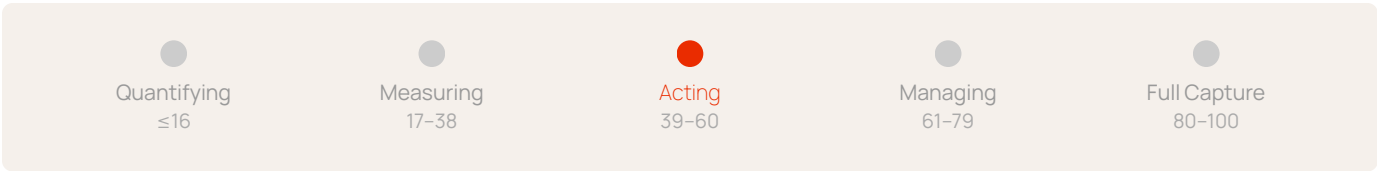
1–2 years in deployment, and your organization is measuring across some domains with real results. The gap is in the domains that haven’t been formally analyzed yet. At this tenure, that’s not an adoption question – it’s a prioritization question. Your organization is actively managing ambient ROI in Quality, but Capacity and Revenue and Workforce remain in early stages. Your biggest opportunity is in revenue – directional signals observed but not formally validated.

FOUR ORGANIZATIONAL DECISIONS

Each domain represents a distinct question. Capacity: did the time ambient saved get redeployed into patient access? Revenue: did documentation quality reach coding and denial management? Workforce: did provider relief translate to measurable retention economics? Quality: did improved accuracy produce downstream clinical and compliance value? These are four separate decisions, four separate measurement programs, four separate conversations with four separate stakeholders.



TOTAL 55 / 100





CAPACITY

Access Decision Made

Did the time ambient saved get redeployed into patient access — or did it disappear?

Recovered time is ambient’s most universal output. The strategic divide is between organizations that measure it and stop — and organizations that assign it a destination. Capacity measures that decision.

WHERE YOU STAND

Your organization is evaluating whether recovered time can be converted to patient access. No structural changes have been made yet.

\$0 deployed. The value at this level is the decision itself — not dollars.

MEASURED IMPACT

Access decision: Evaluating

THE MOVE FROM HERE

Confirmed additional patients per provider per month attributed to recovered time, with access revenue calculated.

IMPACT

Not yet calculated

YOUR INPUTS

Access decision stage Evaluating

WHAT HIGH PERFORMERS DO

The organizations moving fastest from Level 1 to Level 3 share one thing: they scheduled the operational conversation — with a recommendation in hand — before they thought they were ready. The number doesn't need to be perfect. It needs to be in the room.

MATURITY LEVEL

Level 2 of 4



REVENUE

Directional Signal

\$128K
per year

Did documentation quality improvements reach revenue cycle – or stop at the note?

Every encounter is coded. The question is whether it's coded at the specificity your documentation now supports – and whether your revenue cycle team is part of that conversation. Organizations that close that loop find it is worth millions. Most haven't asked the question yet.

WHERE YOU STAND

Your organization estimates 1–3% of encounter revenue is being affected by documentation improvements.

At 32,000 documented encounters × \$200 per visit:
1.0% = \$64K
3.0% = \$192K

This is your team's directional estimate based on trending data. A formal before/after analysis (Level 3) would validate this figure.

3 areas showing movement:
wRVU per encounter trending upward, Denial rates trending downward, CDI query volume decreasing

MEASURED IMPACT

\$64K–\$192K directional estimate

THE MOVE FROM HERE

A formal before/after analysis that produces a number revenue cycle leadership can stand behind.

Estimates based on your inputs. Individual results vary.

CALCULATED IMPACT

\$128K
per year

YOUR INPUTS

Areas showing movement	wRVU trending up, Denial rates trending down, CDI queries decreasing
Directional estimate	1–3%

HOW THIS WAS CALCULATED

[lowEstimate] = 32,000 × \$200 × 1.0% = \$64K
[highEstimate] = 32,000 × \$200 × 3.0% = \$192K

WHAT HIGH PERFORMERS DO

The revenue cycle team almost always finds something when they look. The barrier isn't data – it's the first conversation. Organizations that formalize that conversation within 90 days of deployment typically have a number before their first contract renewal.

MATURITY LEVEL

Level 2 of 4



\$210K
per year

Did provider relief translate into measurable retention economics — or stay anecdotal?

Provider burnout is one of the most expensive problems in health system operations — and ambient documentation is one of the few interventions that measurably addresses it. The value only becomes visible when someone is looking for it.

WHERE YOU STAND

At 5% annual turnover across 60 providers, your organization sees approximately 3.0 departures per year.

Replacement cost: Using AMGA benchmark midpoint: \$350K. Total turnover cost is \$1.05M annually.

Of that, your estimate of 20% attributable to documentation burden represents \$210K — the portion potentially attributable to documentation burden reduction.

MEASURED IMPACT

\$210K in
documentation-attributable
turnover exposure.

THE MOVE FROM HERE

Modeling turnover costs with documentation burden as a contributing factor — connecting provider experience data to retention economics.

All inputs are your organization's data. No external correlation estimates applied.

CALCULATED IMPACT

\$210K
per year

HOW THIS WAS CALCULATED

[annualDepartures] = $60 \times 5\% = 3.0$
[totalCost] = $3.0 \times \$350K = \$1.05M$
[docDriven] = $\$1.05M \times 20\% = \$210K$

WHAT HIGH PERFORMERS DO

Provider experience is the most politically powerful data in a health system — and it's consistently under-quantified. Organizations that formalize this measurement tend to use it in ways they didn't initially plan: recruitment, contracts, board presentations.

MATURITY LEVEL


Level 2 of 4



\$150K
per year

Did improved documentation accuracy produce downstream clinical and compliance value — or stay untracked?

Improved notes are the starting point. The real value depends on whether that improvement reaches CDI, coding, quality reporting, compliance, and future AI initiatives. This domain measures how far downstream the documentation improvement has traveled.

WHERE YOU STAND

Documentation quality is driving \$150K in annual value across 3 areas:

- CDI — fewer queries, faster turnaround, or improved capture rates
- Quality measures — HEDIS, MIPS, or Stars gap closure improved
- Chart abstraction — less manual effort for registries, research, or reporting

Some areas have measured data.

Not yet connected:

- Coding accuracy — specificity improvements or denial reduction
- Prior authorization — documentation supporting faster or higher approval rates
- Risk adjustment — HCC capture or RAF accuracy improved for VBC populations

MEASURED IMPACT

\$150K in downstream quality value

THE MOVE FROM HERE

Documentation quality positioned as a strategic organizational asset — informing quality program design, value-based care, compliance governance, and AI readiness.

Based on your organization's estimate. Estimates based on your inputs. Individual results vary.

CALCULATED IMPACT

\$150K
per year

YOUR INPUTS

Connected areas	CDI, Chart abstraction, Quality measures
Measurement depth	Partially measured
Downstream quality value	\$150,000

HOW THIS WAS CALCULATED

[annualValue] = \$150K (organization estimate)

WHAT HIGH PERFORMERS DO

Documentation quality as a strategic organizational asset is where ambient AI's long-term value gets locked in. Quality program design, VBC strategy, AI readiness, payer negotiations — these all depend on structured, complete documentation.

MATURITY LEVEL



How These Numbers Were Built

Every number in this report can be traced to a specific formula, a specific input you provided, and a specific assumption. The assumptions are listed here so that any figure can be challenged, validated, or updated as you collect better data.

CAPACITY LEVEL 2

- Providers in scope: 60
- Encounters/year: 80,000
- Current utilization: 40%
- Time savings applied: 8 min/encounter (Abridge deployment average)
- Conversion to access: 25% of recovered time converts to new encounters
- Revenue per new encounter: \$150 (adjustable by org)
- Confidence adjustment: 30% reduction applied (estimated data, not confirmed)

REVENUE LEVEL 2

- Providers in scope: 60
- Encounters/year: 80,000
- Method: Benchmark range from Abridge deployments at similar scale (\$4K–\$12K/provider/year)
- Note: Revenue figures are directional. Actual impact depends on payer mix and billing workflow.

WORKFORCE LEVEL 2

- Providers in scope: 60
- Annual turnover rate: 15%
- Replacement cost per provider: \$350K
- Formula: Departures × Replacement Cost (full attribution, conservative baseline)

QUALITY LEVEL 3

- Providers in scope: 60
- Readmission reduction: 0.3–0.8% of applicable discharges
- CMS penalty avoidance basis: HRRP program benchmarks
- Note: Quality figures require confirmed outcome tracking data to move from directional to validated.

The organizations that move from Acting to Level 3+ across multiple domains in under 12 months don't do it alone.

What they have in common is that the domains they've measured have given them leverage. The signal you've built is real. The organizations that move quickly from here use that signal to accelerate the unmeasured domains — not one at a time, but as a connected program. The measurement infrastructure you've started is the hardest part to build from scratch. You're not starting from scratch.

At 1–2 years, the urgency is real. Every month the measurement infrastructure doesn't exist, value that's already there goes uncounted.

WHAT THE PATTERN SHOWS

TOP PRIORITY DOMAIN

Capacity
Capacity has measurement underway but hasn't been pushed to validated, actionable impact. The infrastructure is in place. The next move is pushing it to Level 3.

WHAT HIGH PERFORMERS SHARE

- They've moved beyond measuring one domain in isolation — they track how Capacity, Revenue, and Workforce interact.
- Long-tenure organizations find that ambient adoption stabilizes faster — providers aren't unlearning a workflow, they're replacing a burden.

THE PATTERN AT YOUR SCORE

Acting
Some domains are generating confirmed value. The ones that haven't been measured yet are the highest-return opportunities — because the infrastructure to measure them already exists.

YOUR NEXT MOVES

- 2

Capacity

TOP PRIORITY

Complete a patient volume measurement that confirms how many additional patients are actually being seen per provider per month.
- 2

Revenue \$128K

Complete a formal before/after analysis that produces a dollar number revenue cycle leadership can stand behind.
- 2

Workforce \$210K

Model turnover costs with documentation burden as a contributing factor. Connect provider experience data to retention economics.
- 3

Quality \$150K

Position documentation infrastructure for value-based care — supporting risk adjustment, population health reporting, and payer negotiations.

WHAT THIS ASSESSMENT REPRESENTS

Most vendors show you a number. This assessment shows you a measurement architecture — four domains, four levels of rigor, each with a defined formula and a defined gap between where you are and what full capture looks like. The organizations that reach Level 3+ across multiple domains in 12 months don't do it because they had a better deployment. They do it because they built a structured measurement program around what the deployment was producing. That program has a methodology. The methodology is what this assessment is built on.

That's the conversation Abridge is built to have.

Confirmed values are derived from your stated inputs using Abridge deployment methodology. Level 1 domain figures are benchmark ranges from health systems of comparable size — not projections for your organization. Actual results depend on execution, organizational readiness, and market conditions. Assessment date: March 19, 2026.